
PERRY AND THE CARDS
#1
IDIOT
NSAID
the FLAMING HEART
DIALYSIS fountain
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2.6 - Acute Pericarditis SOAP

1) Flaming guitar case:
Acute pericarditis = inflammation of the fibroelastic sac surrounding the heart (i.e., pericardium)

2(S) Shark tooth necklace:
Pleuritic chest pain = most common symptom of acute pericarditis **(Chief Complaint)**

3(S) Guitar player leaning forward:
The chest pain of acute pericarditis is classically improved by sitting up and leaning forward **(HPI)**

4(S) Flame headband:
Patients with an infectious cause of acute pericarditis may also present with **fever (HPI)**

5(S) Idiot sign on guy holding #1 foam finger:
The vast majority of cases of acute pericarditis are **idiopathic** → etiology is often presumed to be viral **(HPI)**

6(S) Viral icosahedron light:
The vast majority of cases of acute pericarditis are idiopathic → etiology is often presumed to be **viral (HPI)**

7(S) Cancer Crab:
Malignancy = most common non-idiopathic cause of acute pericarditis **(PMHx)**

11(S) Lupus Wolf:

Collagen vascular disease, especially SLE, can cause acute pericarditis **(PMHx)**

12(S) Radiation symbol:
Mediastinal radiation can cause acute pericarditis **(PMHx)**

14(O) Nail marks:
Pericardial friction rub = classic exam finding in acute pericarditis → leathery, squeaking sound best heard over left sternal border **(Cardiac Auscultation)**

15(O) White birds above stage:
Laboratory signs of inflammation (e.g., **elevated WBC count**, ESR, CRP) are common in patients with acute pericarditis **(Labs)**

16(O) Test tube-shaped wind chime:
Laboratory signs of inflammation (e.g., elevated WBC count, **ESR**, CRP) are common in patients with acute pericarditis **(Labs)**

17(O) Elevated **C**a**RP**et:
Laboratory signs of inflammation (e.g., elevated WBC count, ESR, **CRP**) are common in patients with acute pericarditis **(Labs)**

20(O) ECG grid sheet music:

Classic ECG changes in acute pericarditis = diffuse ST elevation and PR depression in all leads EXCEPT right-sided leads (i.e., aVR, V1) **(ECG)**

21(O) Many ST street signs:
Classic ECG changes in acute pericarditis = **diffuse ST elevation** and PR depression in all leads EXCEPT right-sided leads (i.e., aVR, V1) **(ECG)**

22(O) Fallen PR manager:
Classic ECG changes in acute pericarditis = diffuse ST elevation and **PR depression** in all leads EXCEPT right-sided leads (i.e., aVR, V1) **(ECG)**

23(O) Falling ST street sign on right side:
Classic ECG changes in acute pericarditis = diffuse ST elevation and PR depression in all leads **EXCEPT right-sided leads** (i.e., aVR, V1) **(ECG)**

25(O) Heart-shaped megaphone:
Order an echo in patients with suspected acute pericarditis to check for pericardial effusion **(Echo)**

26(O) Guitar case filled with water:
Order an echo in patients with suspected acute pericarditis to check for **pericardial effusion (Echo)**

27) DDx clipboard:
For further workup of Acute Pericarditis, see Chest Pain DDx

9(S) Dress with many moons:
Post-MI (aka **Dressler**) syndrome occurs weeks/months after MI ← results from immunologic phenomena **(PMHx)**

10(S) Antibody-shaped designs:
Post-MI (aka Dressler) syndrome occurs weeks/months after MI ← results from **immunologic phenomena (PMHx)**

30(P) ASA umpire:
NSAIDs (typically **aspirin**) & colchicine are also used in treatment of Dressler syndrome **(3)**

28(P) NSAID fire extinguisher:
NSAIDs = first-line therapy for acute idiopathic/viral pericarditis **(1)**

29(P) Backup choir singers:
Colchicine = first-line therapy for acute idiopathic/viral pericarditis (combined with NSAIDs) **(2)**

13(S) BUN bag:
Uremic pericarditis affects patients with advanced renal failure (i.e., BUN > 60) **(PMHx)**

32(P) Dialysis fountain:
Perform urgent hemodialysis in patients with uremic pericarditis **(5)**

24(O) Blank ECG grid wax paper:
Classic ECG changes are NOT seen in uremic pericarditis **(ECG)**

18(O) T-bone steak:
Elevated troponins in acute pericarditis are suggestive of myopericarditis **(Labs)**

19(O) The Flaming Heart:
Elevated troponins in acute pericarditis are suggestive of **myopericarditis** **(Labs)**

8(S) 3 suns:
Peri-infarction pericarditis occurs within 3 days after MI ← results from acute local inflammation associated with MI **(PMHx)**

31(P) NO fire extinguisher here:
Avoid NSAIDs in patients peri-infarction pericarditis as it can impair scar formation & predispose to ventricular free wall rupture **(4)**